

TeleRehab

Assignment-1 #G302
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Letter of Transmittal

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Subject: Submission of CSIT226 Human Computer Interaction – Assignment 1 Report

Dear Dr., Soly and Dr. Abdalla,

It is our great pleasure to submit the **Assignment 1 Report for CSIT226 – Human Computer Interaction**, for your kind review and evaluation.

This report was prepared with due adherence to the specified format of assignment reports and follows a structured format of a professional report with a letter of transmittal, executive summary, introduction, body sections, conclusion, references, appendices, and a report on task distribution among group members.

This report highlights our chosen problem domain and our proposed systems following our research in the domain and established norms of Human-Computer Interaction.

Under the body of the report, the aspects of usability goals, design principles, cognitive, social, emotional aspects of interaction, selection of interface type, data collection methods, use of sample or dummy data, data representation, analysis of data, and use of low-fidelity prototypes as wireframes have been included.

Although we are still in the learning curve, through this assignment, we were able to derive important insights regarding user-centric design, usability evaluation, and overall implementation of HCI concepts into problem domains.

We hope that this report meets the requirements of this assignment and indicates our level with regards to this course material. We also look forward to your feedback to help us produce even better work in future stages of this project.

Thank you for your time and consideration.

Yours sincerely,
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Executive Summary

This report discusses the design of TeleRehab and how it follows user centered design. TeleRehab is a digital rehabilitation system for physiotherapists and patients that aims to simplify the process. Other solutions in the market are overly complicated, unintuitive and frustrating to navigate.

To find the features required, interviews and user surveys were conducted. These were used to find the features that the users needed, what they struggled with and the features they wanted for rehabilitation. The results were used to find key features needed, like usability goals, interface types, interaction types and interface types.

From our research, TeleRehab decided to use a multi-platform solution for its users. It supports mobile application, web based and wearable integration. TeleRehab provides a clear guidance for how to do exercises and shows progress in a neat way. The system is kept basic, simplifying interaction so any user can use the application easily.

In conclusion, the proposed TeleRehab systems shows the importance of a user centered design to ease users into online rehabilitation. The platform increases usability, simplifies complexity and provides an overall better experience than other options.

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Introduction

TeleRehab is a better way to rehabilitate patients. It not only provides an easy interactive system for patients to rehabilitate more easily but also helps track their progress for physiotherapists. Existing applications have hard to read and complicated systems, especially for patients. They also either don't have proper support for different platforms or aren't properly supported. To counter these issues, we will develop an easy to use and accessible application. The application will also have proper tracking, clear graphs and will meet the local safety requirements.

Usability Goals

Effectiveness

- Specific exercise videos provided for specific targeted patients with specific needs, for example patients with post-surgery get videos on how to fully recover within 30 days and stay in shape.
- The platform has ai motion tracker which can detect in real time if the patient's form is right. It also ensures that the patient exercises properly so that maximum output can be gained from the exercise.
- The platform provides step-by-step visual and verbal guidance on how to work on with the complex movements and turn it into a manageable way so that the patients could exercise properly.
- A therapist dashboard has been provided for physiotherapists to track and monitor patient's movements and their progress remotely for them to adjust their treatments.
- The platform also provides pain tracking in which the platform could detect if the patient is feeling any pain so that it helps the clinician to identify issues early and treat them.

Efficiency

- Exercise reminders are automated so that it can reduce the time spent by patients to remember it.
- Eliminates the time wasted on mobile apps by patients accessing quick access which can search the exercise in an instant.
- Some of the exercises are pre-configured so that the therapists can easily assign it to the patients with common conditions.
- The platform is multilingual and has many languages, so this allows the patient to access the platform with ease without any inconvenience.
- Progress is presented in visual data so that the therapist can quickly access the patient's status without any delays.

Safety

- Form correction AI makes sure that the form is correct, reducing the risk of injury during exercises.
- Pain tracker ensures patients can report any discomfort immediately, which helps therapist to adjust program before any injuries can occur.
- Caregiver portal is available on the platform which can help family members to track the progress of the elderly or dependent patient so that it prevents any risky exercises.
- Session confirmation pop up message is shown and it ensures that the patients are ready for high intensity exercise and in a safe environment.
- If a patient misses a particular activity the therapist can look at them and give them a gentle reminder also reschedule the date for them.

Utility

- Progress analytics with charts and trends both for the patient and therapists.
- Riayati integration for access to medical records.
- Exercise history and logs.
- Offline exercise for patients with less network coverage.
- Video consultation with therapists.
- Wearable sensors to integrate the movement of the user when exercising.

Learnability

- Video demonstration on how to do exercises and audios to help patients understand better.
- Simple user interface for navigation so that elderly can access the platform with ease.
- Practice mode in the platform for rehearsals before actual exercise.
- Help tools in app aid when there is update in the system and user notices them.

Memorability

- The color code on the website is well organized, for example green for completed exercise and orange for incomplete exercises.
- Recently completed exercise come at the top of the list.
- 2-3 taps in navigation can take you anywhere in the platform this reduces memory load.
- Same pictures and hospital icons that are presented in real life are also used in the platform for easy identification.
- The reminder for today's exercise pops up when the platform opens for easy navigation.

Design Principles and Conceptual Model

Principle	Implemented	Justification
Visibility	YES	The main actions like Start Exercise, Next Step, Pain Level, Progress are clearly visible on the screen. Progress bars, exercise timers and body guides always help the user see the system's status (Norman, 2013; Preece, Sharp & Rogers, 2019).
Feedback	YES	The system offers immediate feedback in the form of visual cues such as the use of green and red lights for correct and incorrect movement. It also gives audio prompts and vibration in wearable devices, as well as updates on progress after every session. This gives the user confidence that their actions are being correctly recorded (Norman, 2013; ISO, 2019).
Affordance	YES	Buttons are large and high contrast. Sliders and progress bars provide visual cues on how they can be used, making it intuitive even for elderly people (Norman, 2013; ISO, 2019).
Constraints	YES	Users can only select exercises that have been assigned by their therapist, preventing incorrect actions. Input limits, like a pain scale ranging from 1-10, minimize mistakes and ensure correct entries (ISO, 2019; Dubai Health Authority, 2024).
Mapping	YES	Controls are very clear, matching what happens, which makes the app very easy to understand and use. This reduces cognitive load (Preece, Sharp & Rogers, 2019).
Consistency (Internal)	YES	Colours, icons, layouts, and interaction patterns remain consistent across the app. This helps users learn the interface quickly and reduces confusion.
Consistency (External)	YES	The therapist dashboard resembles common hospital charts, and the app utilizes common phone interactions, making it very easy to use.

Error Prevention & Recovery	YES	Clear instructions, confirmation before ending a session, and the ability to redo or pause exercises all help to prevent mistakes. The therapists can review the data to correct the program if mistakes were made during the process (ISO, 2019; World Health Organization, 2019).
Learnability	YES	The app uses common metaphors such as guided workouts, daily streaks and progress charts. It's a intuitive interface which helps first-time users to use the system with minimal or no training (Preece, Sharp & Rogers, 2019).
Accessibility	YES	Large text, voice guidance, multilingual support and wearable-based interaction support users with vision, literacy, or mobility limitations, particularly elderly patients.

Conceptual Model

TeleRehab follows a task-based conceptual model focused on rehabilitation activities rather than system complexity.

- Exercises
- Sessions
- Progress and pain levels
- Feedback and correction
- Clinical review and adjustment

The system is designed so users focus on performing therapy rather than navigating complicated medical interfaces.

Interaction Types

TeleRehab uses a hybrid interaction model (Preece, Sharp & Rogers, 2019).:

1. Direct Manipulation

Used mainly in the patient mobile application:

- Tapping to start exercises
- Movement guidance during exercises
- Visual feedback

2. Conversing

Used in voice guidance and multilingual audio instructions:

- Spoken prompts during exercises
- Voice reminders and encouragement

This type of interaction reduces anxiety and supports users who may struggle with reading or other difficulties. (e.g. elderly patients) (ISO, 2019).

3. Instructing

Used in both patient and therapist interfaces:

- Selecting exercise programs
- Logging pain levels
- Therapists assigning or modifying rehabilitation plans

Benefits of the Design Approach

- Users can quickly understand how the system works (Norman, 2013).
- Design decisions prioritize rehabilitation goals over technical features
- The system supports both beginner and experienced users
- Clinical safety and usability are maintained in accordance with DHA telehealth standards (Dubai Health Authority, 2024).

Cognitive Aspects

Our design for TeleRehab is strongly aligned with cognitive design principles - The Cognitive aspects and their implementations are as follows:

Cognitive Aspect	Supported by TeleRehab?	Evidence
Attention – Directing the user to focus on relevant information instead of distractions	Yes	Guided steps & reminders
Perception – How information is perceived and understood	Yes	Visual & audio cues + wearable feedback

Memory – Ability to encode information (recognition over recall)	Yes	Recognition-based frameworks & activity logs
Learning – Understanding how to use the system	Yes	Active learning & real-time feedback
Reading / Speaking / Listening – Different ways to understand and communicate information	Yes	Multilingual audio & visual support
Problem Solving & Decision Making – Using external data (charts, dashboards) to weigh options	Yes	Dashboards & analytics
Mental Models – Understanding how the system works	Yes	Feedback loops
External Cognition – Using tools to reduce mental effort	Yes	Progress tracking & reminders

Social Interaction

TeleRehab has many features that assist in social interaction between patients and physiotherapists. Using TeleRehab a patient and physiotherapists can communicate using chat messages which can be used for questions or guidance instead of meeting physically.

For patients, TeleRehab has features for sending pain location and levels through the app. This can precisely and clearly show to physiotherapists where the pains are located. Physiotherapists can also send exercises with steps, so patients can safely perform the exercises. The physiotherapist can also monitor the progress done by the patient in exercises. The app also has visual graphs and logs to further ease communication. With all these features, TeleRehab provides a functional, easy to use environment for patients and physiotherapists to communicate easily and achieve their goals.

Emotional interaction

Our TeleRehab app is made in a way that helps people both emotionally and practically. Our app aims to make rehabilitation phase more enjoyable and motivating rather than exhausting, boring and sometimes even painful. For us, helping our users makes us feel inspired and secure while also utilizing the system.

During workouts our app offers encouraging and good feedback like for example: it displays message like “keep going” when the user has done the movement and posture perfectly and, we don’t send critical messages to our users when they do a mistake instead, we send a gentle message which guides them throughout their journey with us.

We have also added progress indicators like progress bar, completion charts and trends in pain level so that the users would feel a sense of achievement and control during recovery stage. The app also features a slight anthropomorphism which is like having a virtual coach so that the users can feel motivated without anyone judging them.

Overall, our design makes the platform more pleasant to use and reduces anxiety and frustration this encourages users to continue using our app and follow the rehabilitation practices.

Interface Type

- Mobile graphical interface for patients

This interface is based on GUI and mobile interface theory, which focuses on icons, buttons, gestures and visual feedback. This interface is simple, visual and easy to use, which is important for patients who need to do their exercises.

- Web-based graphical dashboard for therapists and clinicians

The therapist dashboard follows the dashboard and information visualization theory, presenting complex data clearly and "at a glance".

Therapists have to analyse trends and performance, and this can be done quickly with the help of a dashboard. It can help with decision-making without flooding the therapist with data (Sauro & Lewis, 2016).

- Smartwatches and wearable motion sensors

This interface aligns with wearable interface theory, which focuses on technology embedded in devices worn on the body.

Wearables are able to carry out real-time exercise monitoring all the time when the patient is moving and this is perfect for physical rehabilitation.

- Speech-based interaction for guidance and reminders

The interaction is based on the theory of speech interface, often combined with multimodal interaction (speech, visuals and touch).

Speech interaction is more inclusive because it reduces physical and cognitive efforts, especially for patients who have limited mobility and vision.

These interface choices fit well with the rehab activities and make the technology easier for patients to use.

Data Collection

Why we used these methods:

There are several key reasons we used a mixed approach to gather our data:

1) Balancing quantitative & qualitative data for better insights:

We used a mixed approach involving (semi)-structured interviews and surveys to gather our data

- Questionnaires and Surveys provide numerical, quantitative data (pain levels, comfort levels, frequency, etc) that we can use to statistically analyze and identify trends across groups of users.
- Interviews provide depth, personal experiences, more nuanced challenges that other methods can't gather.

The combination of both data types gives us enough to work with when designing the app to be both data-informed and user-centered

2) Capturing user diversity & accessibility needs:

The questions covered:

- Behavioral Patterns (Loss of motivation, Forgetfulness)
- Physical Needs (How to treat chronic pain, Exercise Frequency)
- Tech Comfort (How was their experience with the app, what type of guidance they'd prefer)
- Demographic (Past rehab experience, Age)
- Feedback (Suggested features)

3) Ethics:

We included ethical statements in both methods, guaranteeing:

- Complete Anonymity
- Participation was voluntary
- Data would only be used for academic purposes
- They could stop the interview at any time (only asked for the interviewees)

4) Flexibility in gathering data:

- Our semi-structured interviews made each interview different in terms of depth, allowing for more personal approaches with different interviewees, giving us unexpected insights & deeper learning opportunities.
- Diverse question-types allowed users to express themselves freely while still providing structured data for academic analysis

5) Identifying real-world factors:

We asked questions on:

- Motivation (What gets your motivation going)
- Experience (How rehab has been up to this point)
- Support (What kind of support someone might need and not know about)
- Challenges in daily life (Do limitations in pain interfere with day-to-day activities)

Questions like these helped us understand how environmental, contextual & personal factors are critical for the app's success. How users would need ongoing support, visual demonstrations, flexible scheduling & more.

Summary:

Our mixed-method approach ensured a comprehensive, user-focused & ethical foundation for gathering data, allowing us to design a tele-rehabilitation app that would be effective, accessible & easy to use for a diverse range of users

Data Collection Method

Questionnaire / Survey

The data was collected using an online questionnaire which was basically a survey. The questions were a mix of close-ended and open-ended questions which aim to find out users' experience with rehab, how painful they were, if they exercise often, and what would they like in a rehab app (Sauro & Lewis, 2016).

Ethical Statement

Participants were informed that:

- Their responses are anonymous
- Participation is voluntary
- Data will be used only for academic design purposes

Sample Questionnaire Questions

Warm-Up Questions

What is your age group ? *

Have you undergone physical rehabilitation before ? *

If yes, then what type of rehabilitation have you done? (Select all that apply)

If no, then have you ever been advised by a doctor or therapist to do exercises at home?

Rehabilitation Needs & Preferences

How would you rate your current pain or discomfort during daily activities?

(If you have no pain, select 1)

How often do you currently perform any form of physical exercise?

Technology & Accessibility

How would you describe your experience with technology? *

How comfortable are you using mobile apps?

Which types of guidance would you prefer in a rehabilitation app?

(Even if you've never used one, you can choose what you think would help.)

Would you benefit from reminders or notifications to complete exercises ?

Where do you prefer doing rehabilitation exercises ? *(you can choose what you think even if it doesn't apply to you)*

Open-Ended Reflection

What challenges do you face when trying to stay consistent with physical exercises?

What features would make a rehabilitation app more helpful for you?

Sample Answers

Age Group	Rehab Before	Pain Level	Exercise Frequency	Tech Comfort	Preferred Guidance	Challenges	Suggested Features
18–30	No	1	Regularly	Comfortable	Visual & Audio	Low motivation	Reminders
31–45	Yes (Injury Recovery)	3	Sometimes	Comfortable	Visual & Audio	Busy schedule	Wearable tracking
46–60	Yes (Chronic Pain)	5	Sometimes	Neutral	Visual & Audio	Difficulty following routine	Guided visuals
18–30	No	6	Rarely	Neutral	Vibration feedback	Forgetfulness	Notifications
31–45	Yes (Post-surgery)	7	Rarely	Comfortable	Text & Vibration	Mobility issues	Stretch programs

The collected dummy data gathered will be used in the next part to analyse user needs, challenges and the features they prefer for the rehab app.

Interviews

Besides the online survey, semi-structured interview was organized to gain a better understanding of the user's rehabilitation routines, personal challenges and their expectations

from a rehabilitation application. It provided them with a chance to express their experiences without limitation and voice issues that couldn't fully be captured by structured survey questions (Preece, Sharp & Rogers, 2019).

Ethical Statement

Participants were informed that:

- Their responses are anonymous
- Participation is voluntary
- Data will be used only for academic design purposes
- They were free to stop the interview at any time.

Sample Interview Questions

1. What has your rehabilitation been like to this point?
2. What typically get your motivation going in terms of working out?
3. Have you ever used a rehab or fitness app? Why or why not?
4. What kind of support do you think someone might not realize they need for rehab?
5. Do limitations in pain, or mobility interfere with your daily activities?
6. What would the ideal rehab app from an appearance standpoint have?

Sample Interview Answers (Dummy Data)

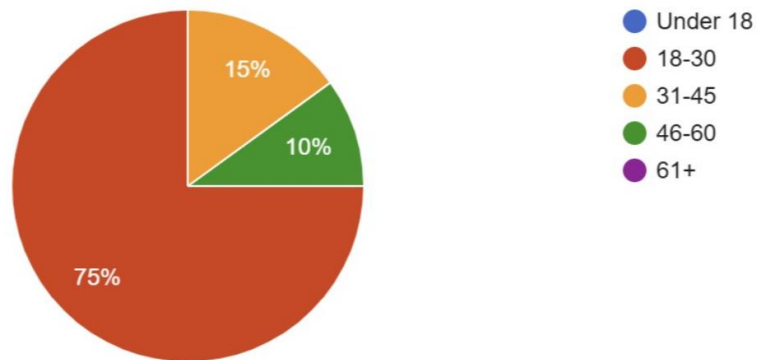
Participant	Key Insights
User A (Post-surgery)	Exercises are confusing without visual demonstrations
User B (Chronic pain)	Stops exercises when pain increases unexpectedly
User C (Young adult)	Loses motivation without reminders or progress feedback
User D (Busy worker)	Needs short and flexible exercise sessions
User E (Older adult)	Prefers slower instructions and simple app navigation

Data Representation and Analysis

Graphs are based on Dummy Data: -

What is your age group ?

20 responses

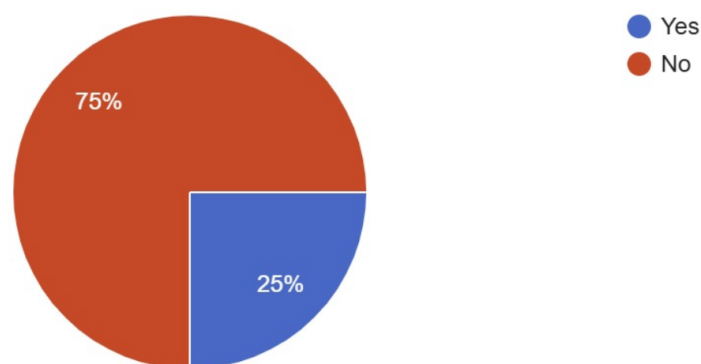


1. The pie chart clearly indicates that most of the respondents are from the age category of 18-30 (75%) this indicates that the most participants are young adults and a smaller portion of 15% could be seen from age category of 31-45 followed a by 10% participation from age 46-60>

Overall, the survey is done by mostly by young adults followed by a small number of elderly people which indicates that app should be friendly to use for both young and elderly.

Have you undergone physical rehabilitation before ?

20 responses

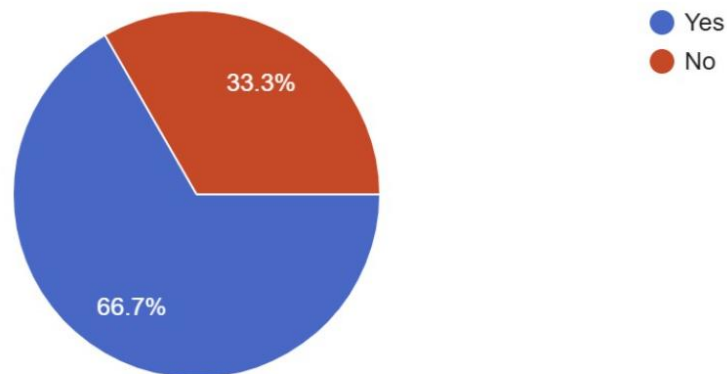


2. The chart shows the history of physical recovery for the participants. The results showed that the majority (75%) had never used physical therapy, while only 25% reported having done so in the past.

This suggests that most clients are most likely new to treatment plans. Due to this, the program should be made easy to use, with clear instructions and support for those who are not familiar with it. This also makes it possible to include encouraging remarks and thorough exercise instructions to assist users feel more comfortable and safer using the system.

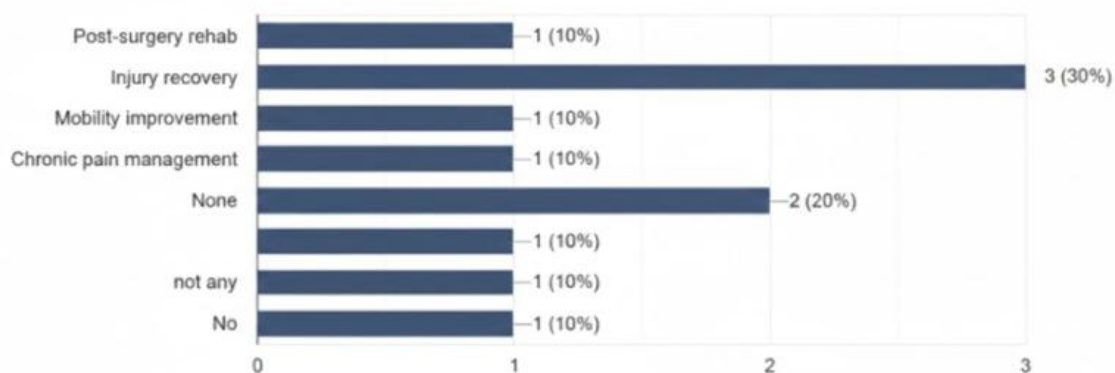
If no, then have you ever been advised by a doctor or therapist to do exercises at home?

18 responses



3. When asked if they have ever received advice from a doctor or therapist to perform exercises at home, the answers are displayed in charts. Most participants (66.7%) selected "Yes," while 33.3% selected "No," according to the data. As evidenced by the fact that most responders were told to complete their exercises at home, there is an obvious need for a system that can directly assist users during home-based rehabilitation.

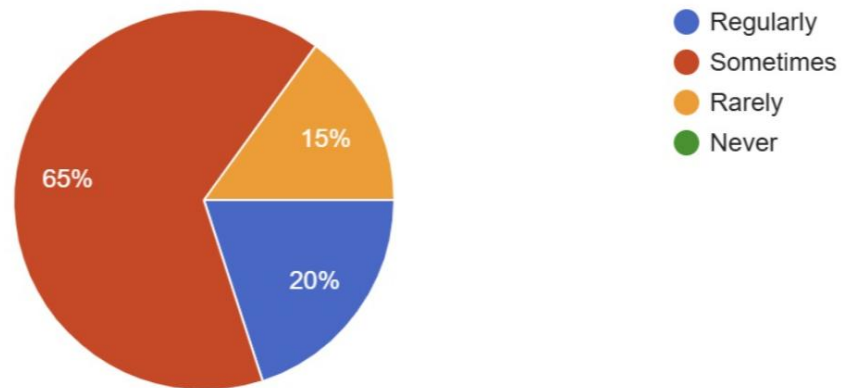
If yes, then what type of rehabilitation have you done? (Select all that apply)



4. The bar chart clearly shows that most of the people (30%) who opt for rehabilitation are the ones who need recovery from their injuries followed by 20% who opted for no rehabilitation while scattered 10 percent goes to post-surgery rehab and chronic pain management and so on. This clearly indicates that a rehabilitation platform is required for most of the cases.

How often do you currently perform any form of physical exercise?

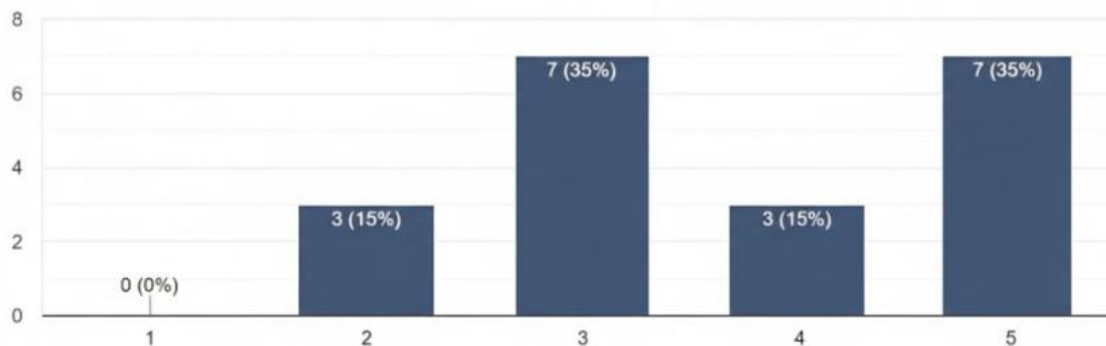
20 responses



5. 65% of respondents indicate that they sometimes do exercises while 20% of respondents regularly exercise followed by a mere 15% of respondents who rarely exercise, which helps us understand that a workout session is required in the rehabilitation platform.

How would you describe your experience with technology?

20 responses

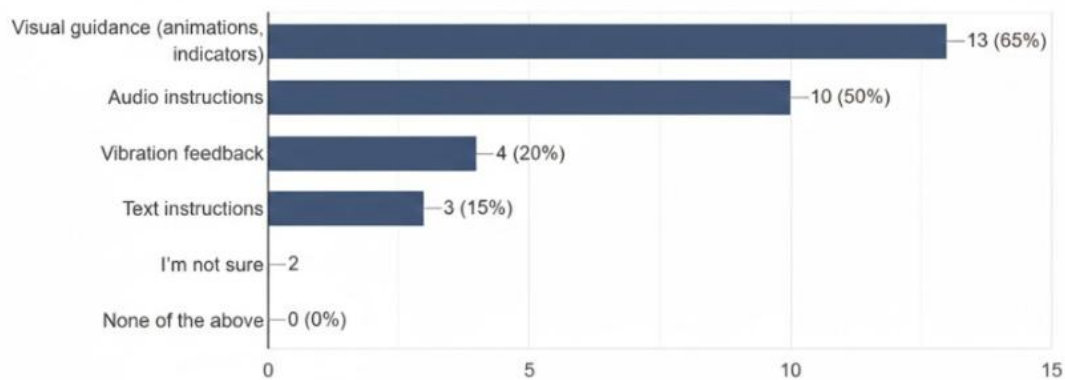


6. Twenty participants completed a questionnaire that measured their self-report level of technology knowledge. Overall, the participants seemed to be relatively comfortable with technology. There was a wide distribution between the highest self-report scores as only 15% of the participants rated themselves at a level 4, the highest score received was 35%, which was evenly distributed at 3 and 5. There was also an evenly distributed 15% with the lowest score of 2 but no participants scored themselves at 1. This shows that all the participants had at least some knowledge of technology so it's easier for us to make it more reliable.

Which types of guidance would you prefer in a rehabilitation app?

(Even if you've never used one, you can choose what you think would help.)

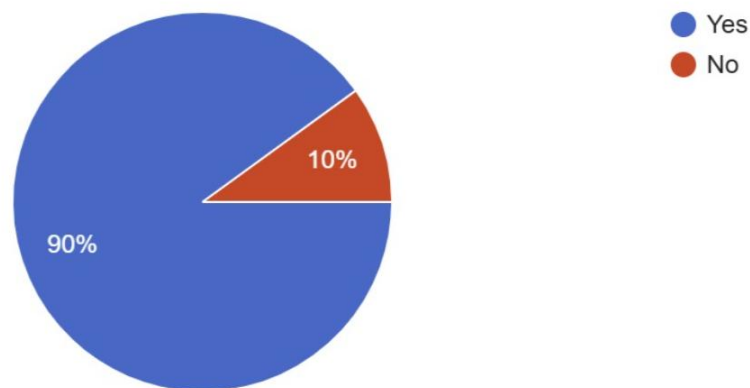
20 responses



7. Several digital guidance was given to users. 65% proved most popular. Audio guidance was the second preferred method 50% Vibration guidance was also selected but very rarely 20% users. Text based guidance was selected even less often 15%. 10% did not know. A note of interest here is that 'None of the above' was not selected this allows us to make changes in the app and bring it to the user needs.

Would you benefit from reminders or notifications to complete exercises ?

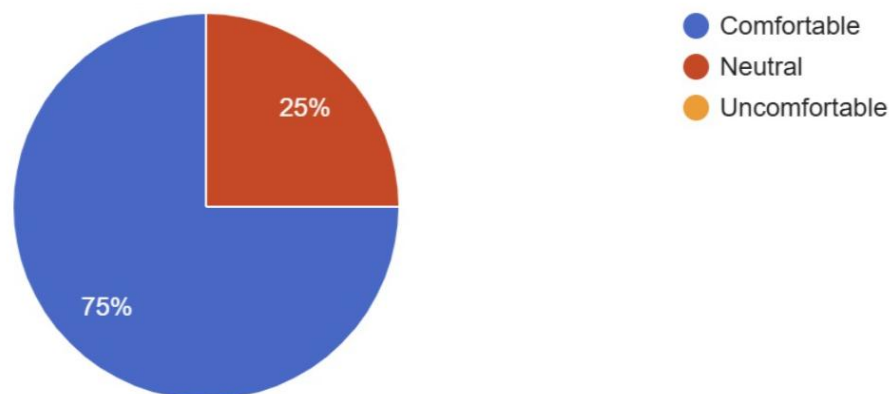
20 responses



8. Reminders are required for most users, as you can see 90 percent of users want notifications while 10 percent do, which clearly states that we need to add reminders for every session.

How comfortable are you using mobile apps?

20 responses



9. 75% of respondents are comfortable using mobile apps which indicates a need for an app for the platform whereas, 25% of respondents chose neutral which indicates they are okay in either way, so this concludes a need for mobile app.

Low Fidelity Prototypes: Wireframes

Web-Based Interface

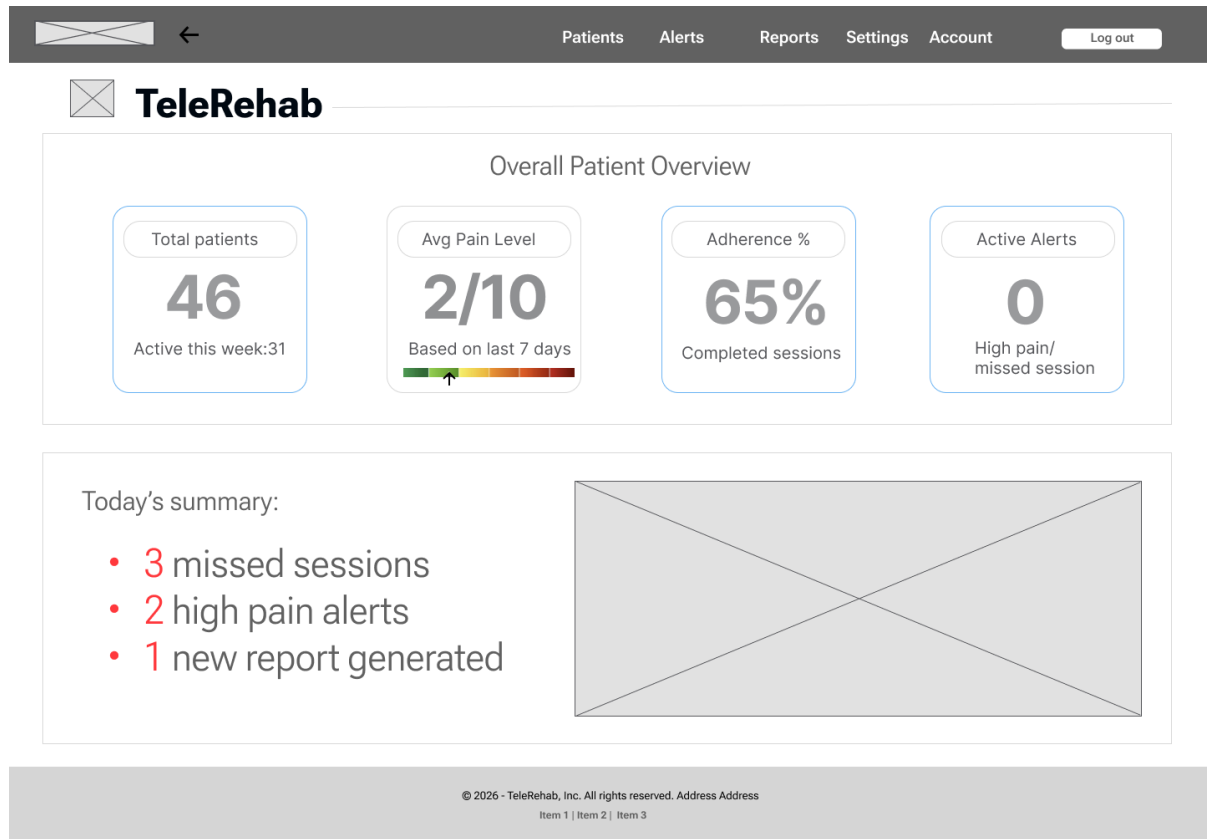


Figure 1. TeleRehab Dashboard Wireframe

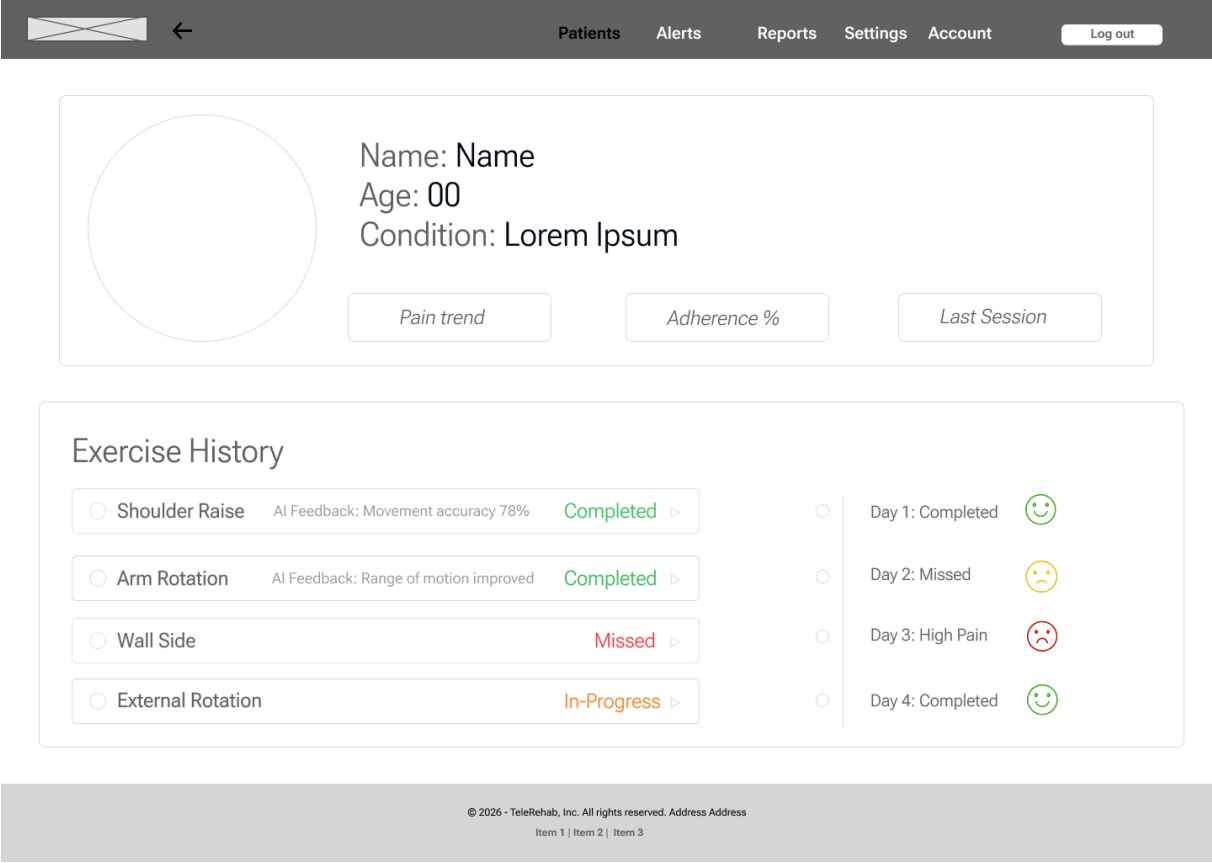


Figure 2. TeleRehab Patient Profile Wireframe

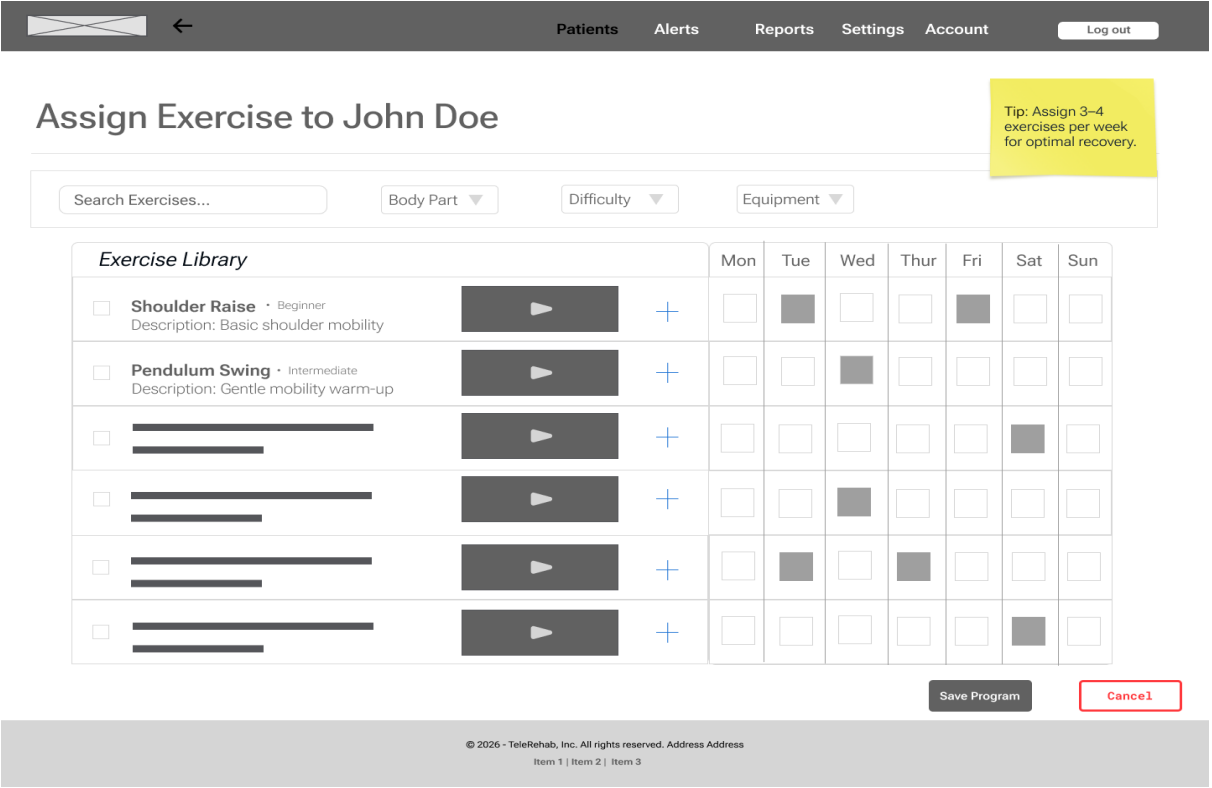


Figure 3. TeleRehab Exercise Assignment Wireframe

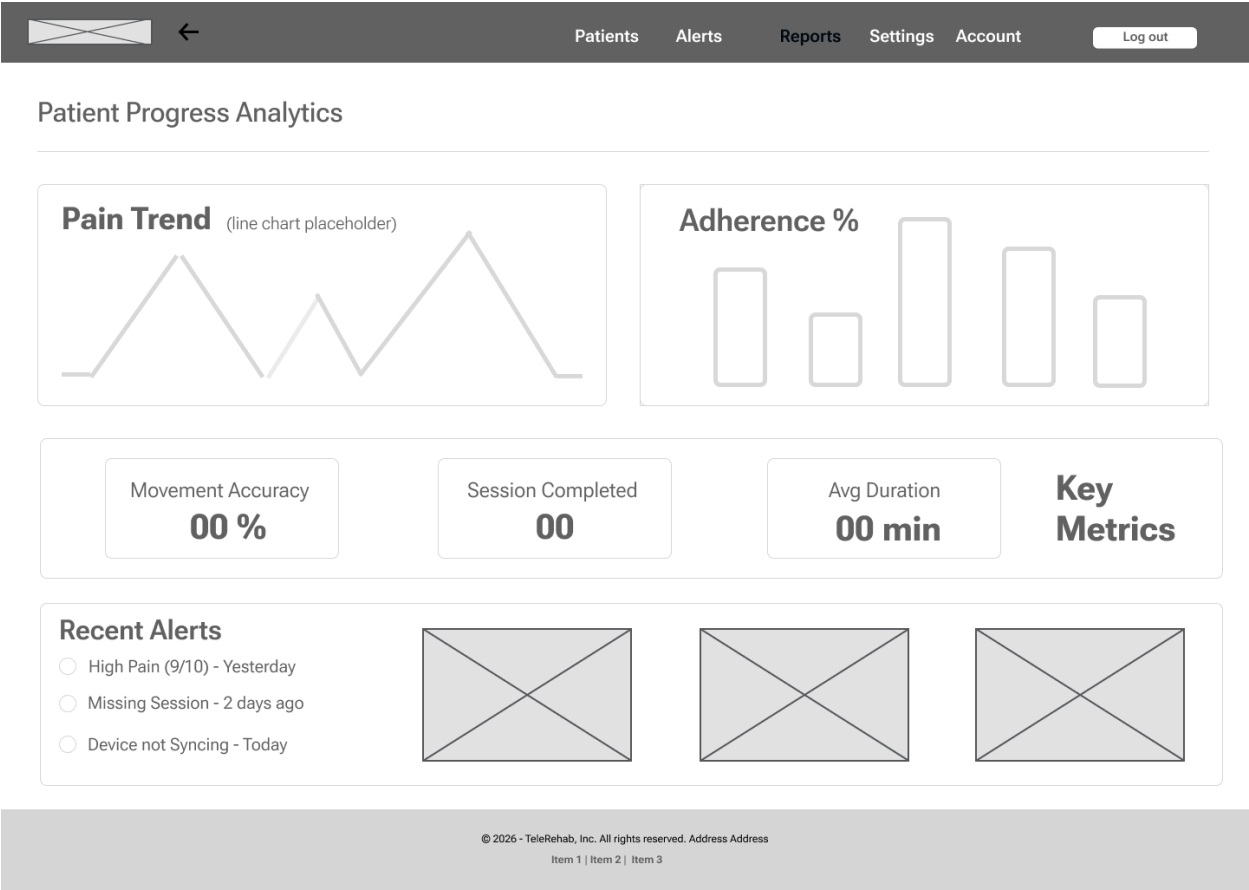


Figure 4. TeleRehab Patient Progress Analytics Dashboard

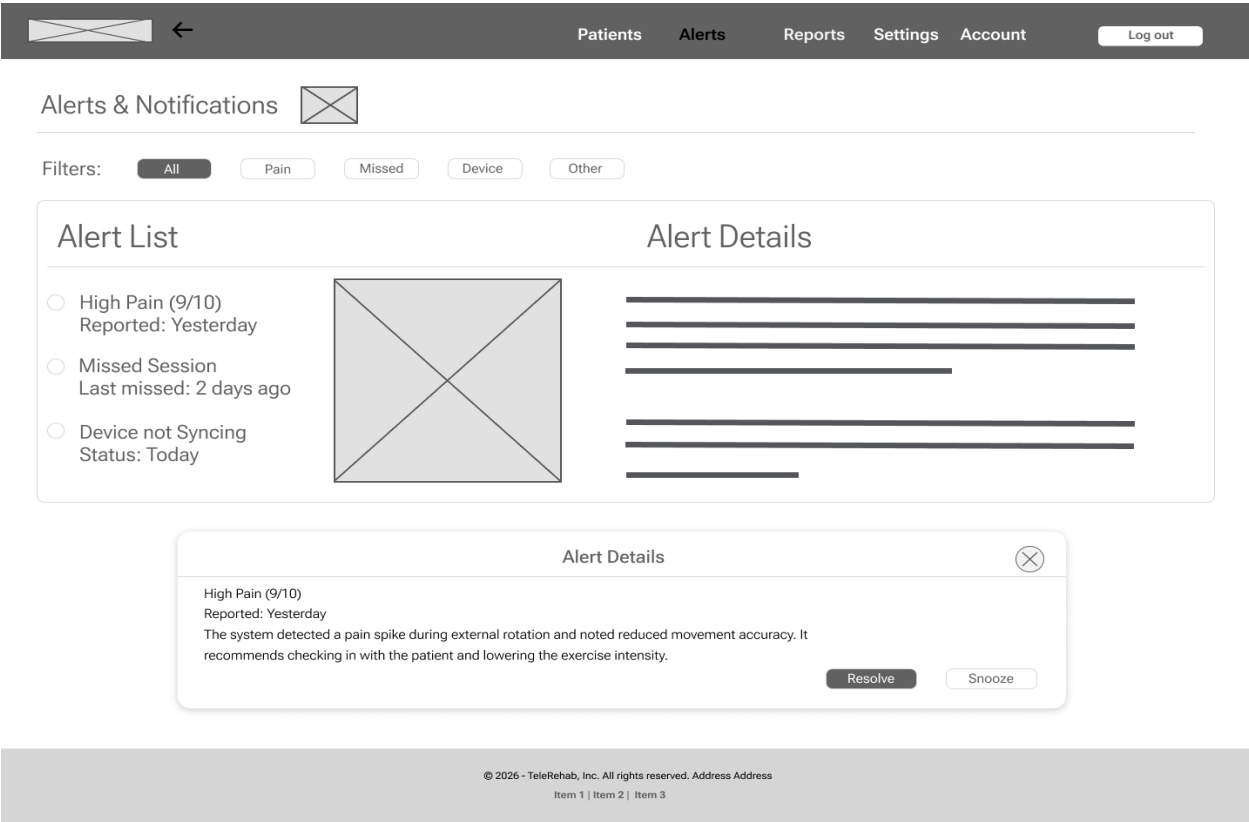


Figure 5. TeleRehab Alerts & Notifications Dashboard

Mobile Interface

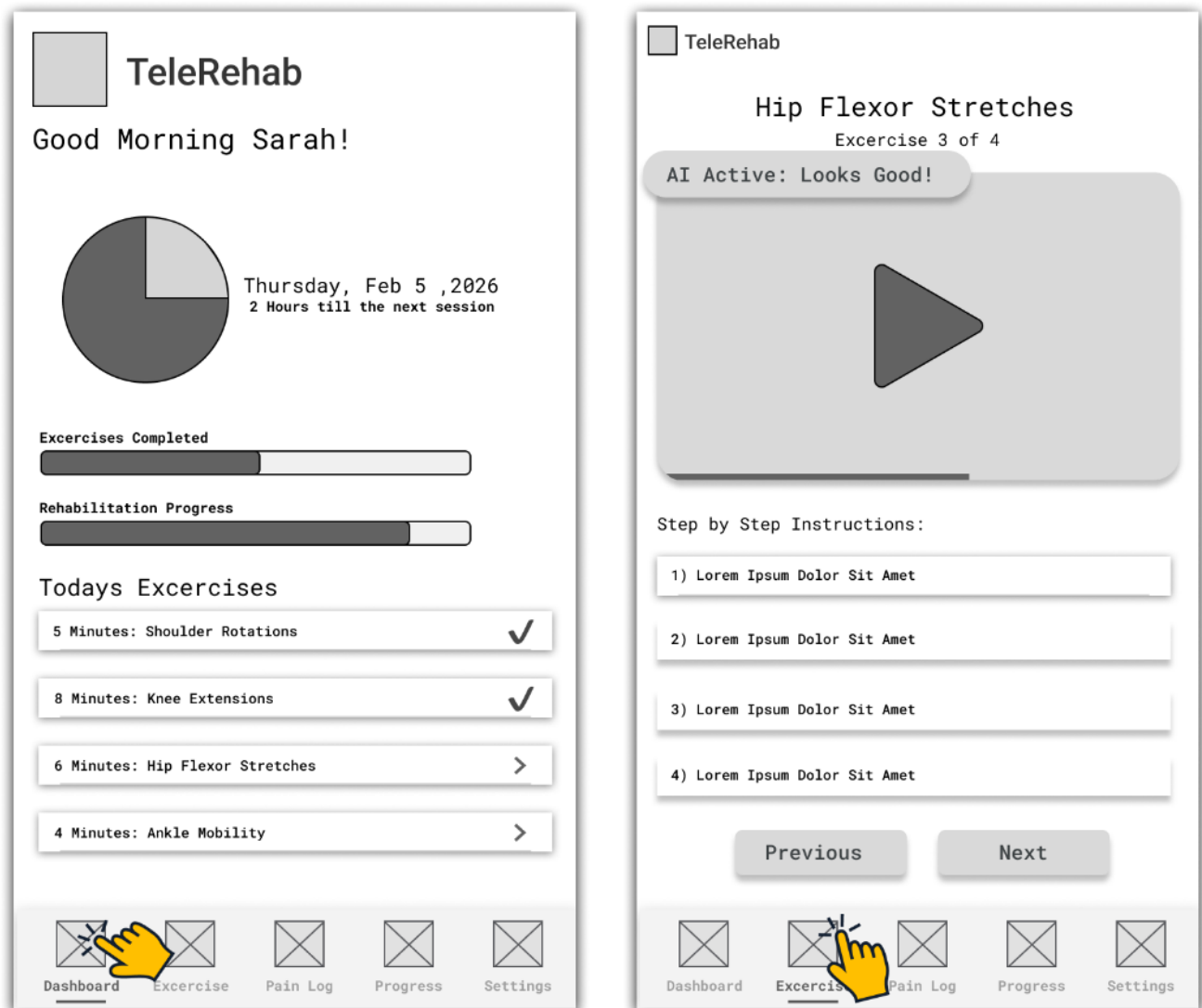


Figure 6. TeleRehab Mobile Dashboard Wireframe

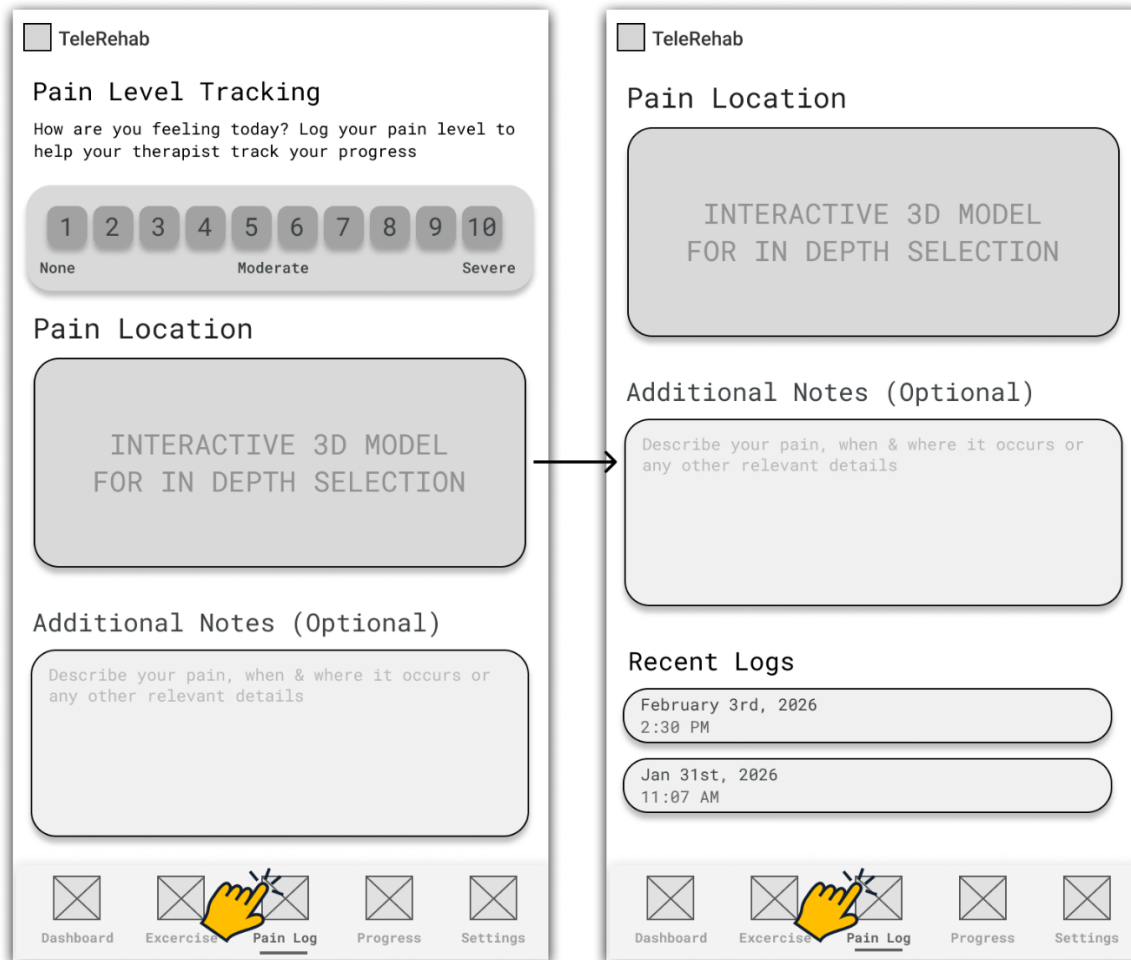
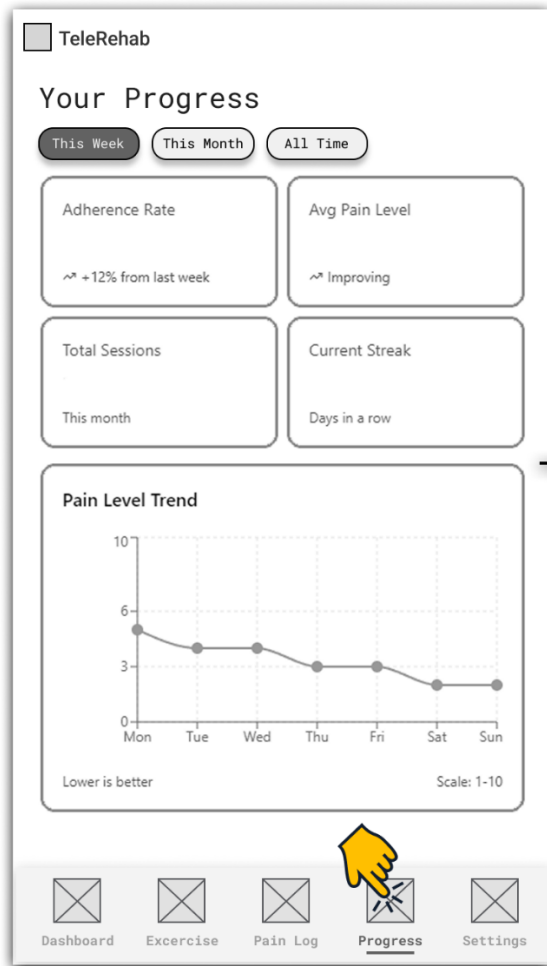


Figure 7. TeleRehab Mobile Pain Log Wireframe



Scroll Down

Figure 8. TeleRehab Mobile Progress Tracking Interface

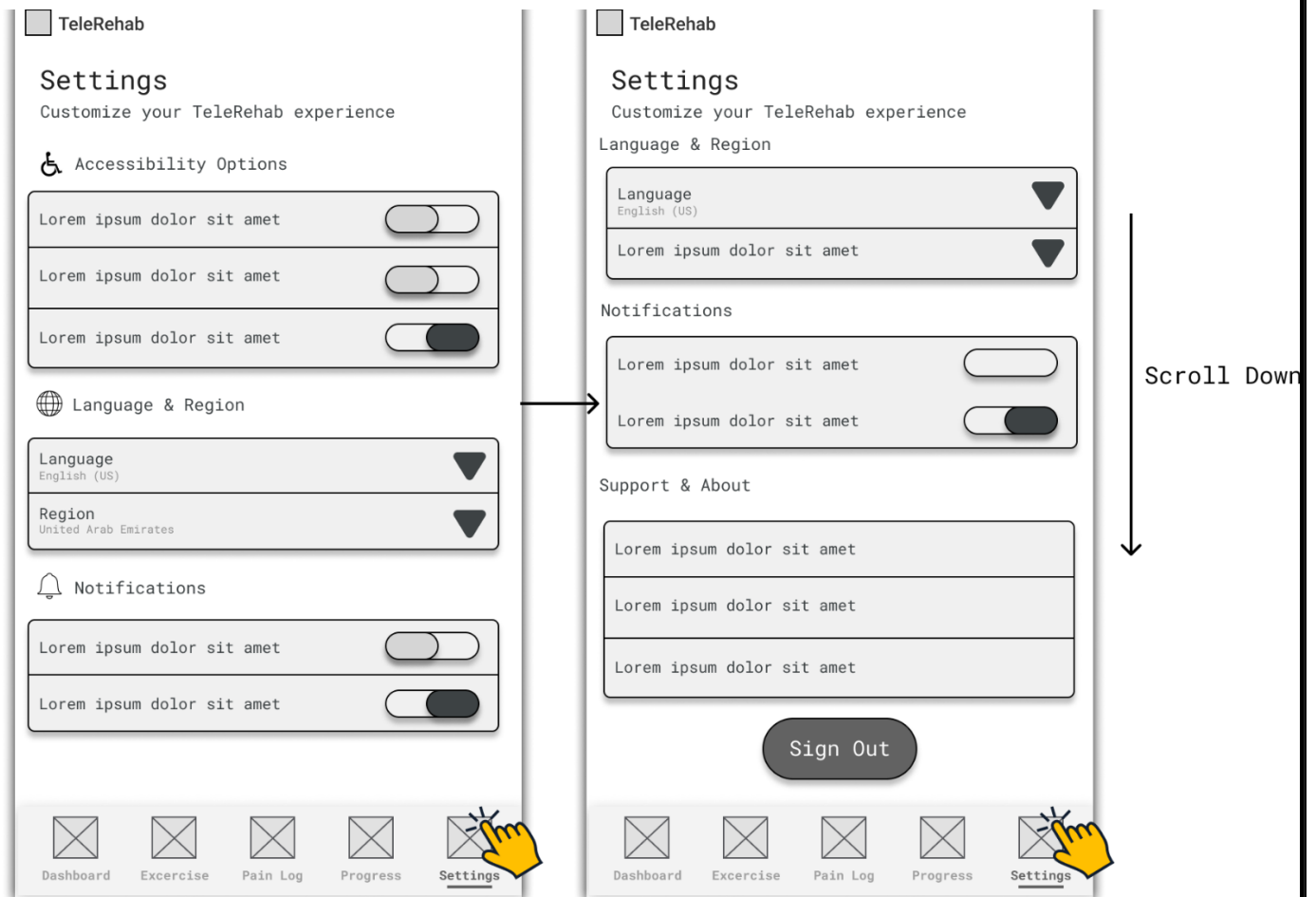


Figure 9. TeleRehab Mobile Settings Interface Wireframe

Smartwatch Interface



Conclusion

In conclusion, TeleRehab is aimed at improving accessibility, usability and safety for users which other platforms do not provide at the required level. The system provided in this report provides an easy system for a patient's rehabilitation journey. It also enables physiotherapists to monitor the progress of a patient in a simple way.

TeleRehab is based on user research which were done through user surveys and interviews. The research pointed out features the application needed for its users. As a result of the research, it helped develop reminders, wearable integration, simplified user interface and more. Visibility, feedback, consistency and error prevention design principles were used to make it usable even for the elderly or for users who aren't comfortable with technology.

In essence, this report provides the importance of TeleRehab and why it is needed. It shows design principles required for such an application and how it impacts user experience with rehabilitation.

References

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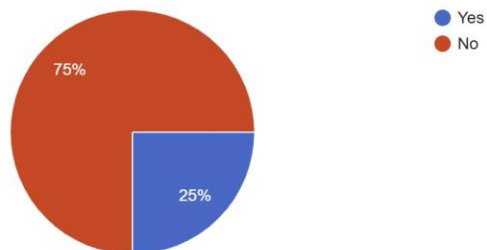
Appendices

Appendix A: Survey/Questionnaire

Have you undergone physical rehabilitation before ?

 Copy chart

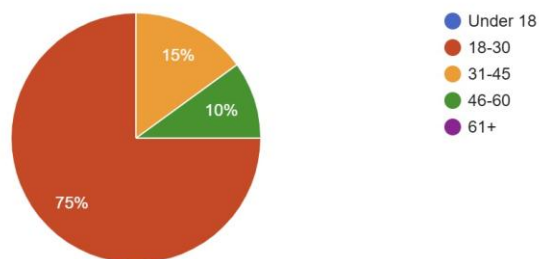
20 responses



What is your age group ?

 Copy chart

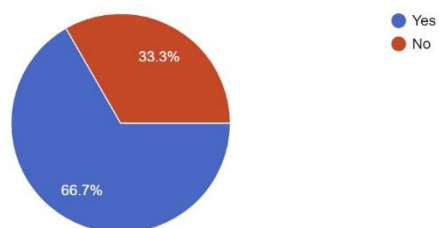
20 responses



If no, then have you ever been advised by a doctor or therapist to do exercises at home?

 Copy chart

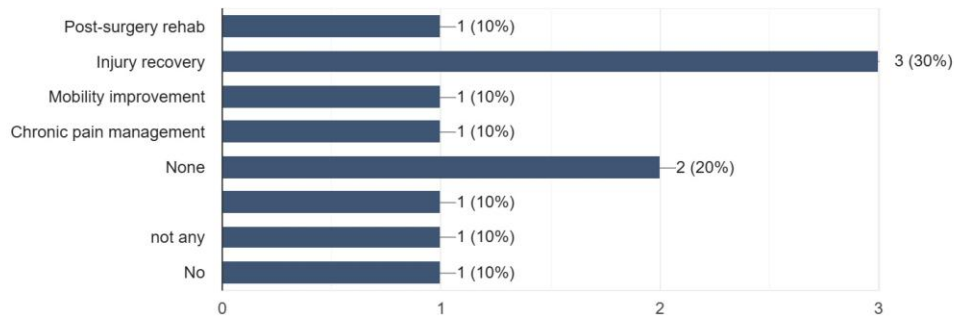
18 responses



If yes, then what type of rehabilitation have you done? (Select all that apply)

[Copy chart](#)

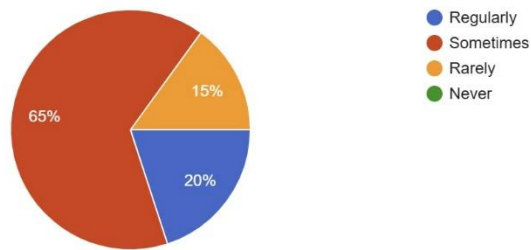
10 responses



How often do you currently perform any form of physical exercise?

[Copy chart](#)

20 responses

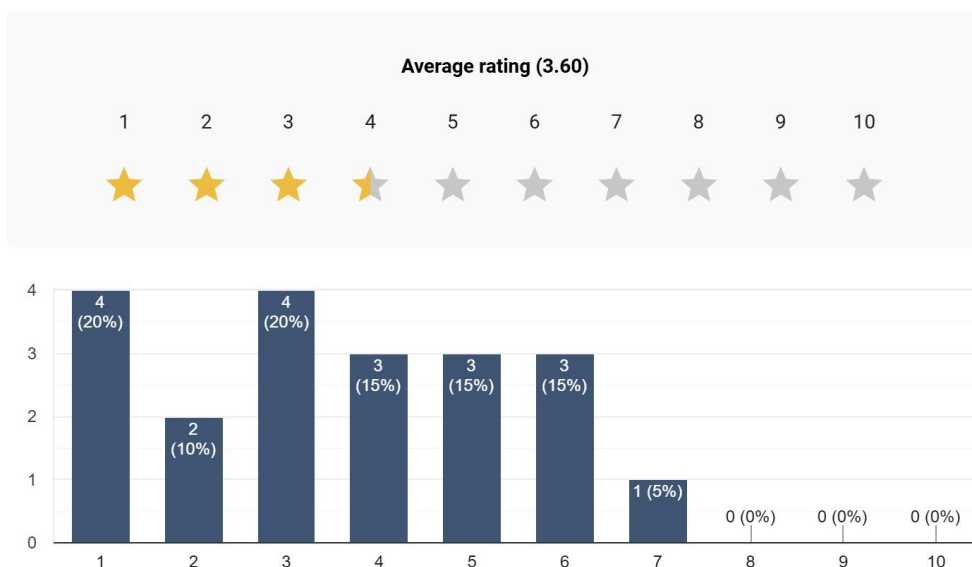


How would you rate your current pain or discomfort during daily activities?

(If you have no pain, select 1)

[Copy chart](#)

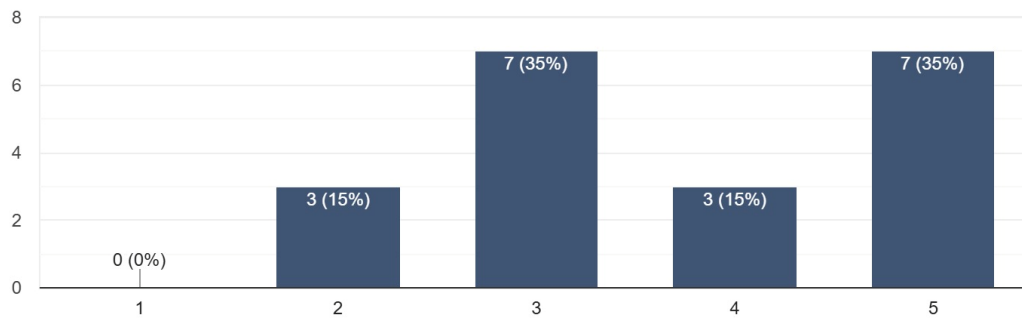
20 responses



How would you describe your experience with technology?

[Copy chart](#)

20 responses

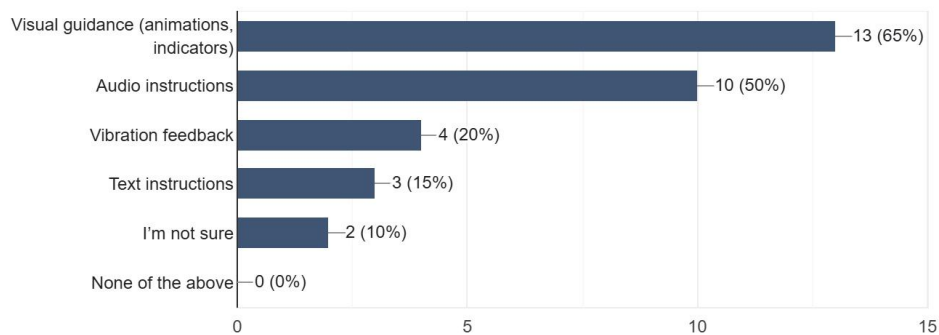


Which types of guidance would you prefer in a rehabilitation app?

[Copy chart](#)

(Even if you've never used one, you can choose what you think would help.)

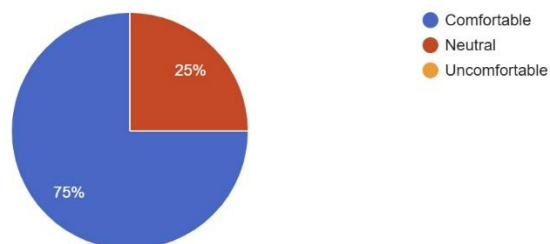
20 responses



How comfortable are you using mobile apps?

[Copy chart](#)

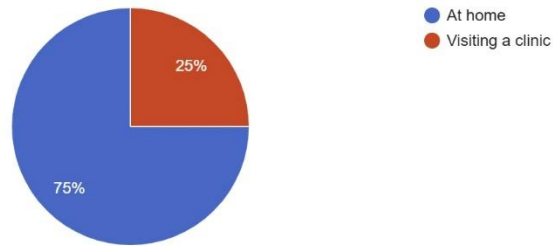
20 responses



Where do you prefer doing rehabilitation exercises ? (you can choose what you think even if it doesn't apply to you)

 [Copy chart](#)

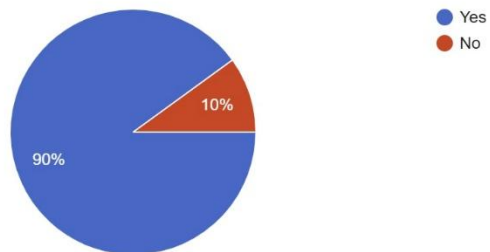
20 responses



Would you benefit from reminders or notifications to complete exercises ?

 [Copy chart](#)

20 responses



What challenges do you face when trying to stay consistent with physical exercises?

18 responses

Laziness and having a busy time schedule

The difficulties in joint pains due to lack of stability which I'm trying to improve on daily basis.

Non

Laziness , lack of motivation

mobility and tight body

Motivation

pain

Forgetful

Planning

What features would make a rehabilitation app more helpful for you?

15 responses

A tracker

Reminder

Easier access

more stretches for more ignored body parts like hips, glutes, chest, and knees

A specialised AI bot

Daily motivation

Progress visualisation

Wearable device support to track my movements accurately.

Tutorials

Work Distribution Sheet

Student Name: _Motasim Saeed_____

Student ID: _8971614_____

Features worked on:

Executive Summary, Title Page, Table of Contents, Introduction; Social Interaction; Conclusion; Appendices (sample data)

Student Name: _Fawaz Rahman_____

Student ID: _9109961_____

Features worked on:

Usability Goal; Emotional Interaction; Data Representation & Analysis; References

Student Name: _Afra Ashraf_____

Student ID: _8957368_____

Features worked on:

Design Principles; Interface Type; Sample of Data (dummy); Low-Fidelity Prototype, Letter of Transmittal

Student Name: _Mohammed Bilal_____

Student ID: _8521372_____

Features worked on:

Cognitive Aspects; Data Collection Method Deployed; Low-Fidelity Prototype; Style

Student Name: _All Members_____

Student ID: _____

Features worked on:

Report on Distribution of Tasks